

ADULT (over 18 years old) VANCOMYCIN PRESCRIPTION CHART

- Prescribing:** Prescribe vancomycin on the patient's main / ePMA prescription chart and document in dose section / qualifier 'Give as per vancomycin chart'.
- Administration:** Infusion fluid: Sodium Chloride 0.9% or Glucose 5%. Maximum infusion rate 10 mg/minute.
- Dosing:** A loading dose is recommended to achieve target vancomycin trough concentrations serum levels.

1 Loading dose: Loading dose is based on patient's actual body weight, independent of renal function and age.

Loading dose	< 60 kg	60 – 90 kg	> 90 kg
	1 g in 250 mL over 100 minutes	1.5 g diluted in 500 mL over 150 minutes	2 g in 500 mL over 200 minutes

2 Maintenance dosing: The maintenance dose is based on the patient's calculated creatinine clearance (CrCl)

Compare patient's actual body weight with table 1. If patient weighs more than the adjusted body weight for their height then the adjusted weight needs to be used when calculating CrCl. **Do not use eGFR from iLab**; use the CrCl calculator on antibiotic website. For patients receiving renal replacement therapy, or are a renal transplant patient, discuss with ward pharmacist or specialist.

Table 1. Adjusted Body Weight (kg)

Height (feet. inches)	Men	Women
5.0	60	55
5.1	63	57
5.2	66	60
5.3	68	63
5.4	71	66
5.5	74	68.5
5.6	77	71
5.7	79	74
5.8	83	77
5.9	85	80
5.10	88	82
5.11	90	85
6.0	93	88
6.1	96	90
6.2	99	93
6.3	101	96
6.4	104	99
6.5	107	102

Table 2. Maintenance doses

Calculated creatinine clearance (mL/minute)*	Maintenance dose	Time after loading to start maintenance dose (hours)	Infusion fluid volume for each dose	Duration of infusion for each dose
Greater than 110	1.5 g BD**	12	500 mL	180 min
90 – 110	1.25 g BD	12	250 mL	125 min
75 – 89	1 g BD	12	250 mL	120 min
55 – 74	750 mg BD	12	250 mL	75 min
40 – 54	500 mg BD	12	100 mL	60 min
30 – 39	750 mg OD	24	250 mL	75 min
20 – 29	500 mg OD	24	100 mL	60 min
10 – 19	500mg every 48 hours	48	100 mL	60 min
Oliguric, anuric or Less than 10 mL/min	Check levels after 48 hours, ONLY once level less than 15 mg/L dose with 1 g	Only once levels <15 mg/L	250 mL	120 min

* Continue to monitor CrCl throughout treatment. Seek pharmacist advice if U+E derange.

** Patients less than 45 kg should be given a maximum starting dose of 1.25 g BD

3 Monitoring

If patient is oliguric, anuric, or has creatinine clearance less than 10 mL/min:

- Check levels after 48 hours. Do not give a dose until the level is less than 15 mg/L.

If patient's creatinine clearance greater than 10 mL/min AND urine output is greater than 0.5 mL/kg/hour:

- A pre-dose level should be taken **immediately before the third or fourth dose**.
- The dose should be given after the pre-dose level is taken if serum creatinine is stable with good urine output
- Give time of last dose and time sample taken, details of dose and latest serum creatinine on the sample request form (without which the result cannot be interpreted)
- Normal target range is 10 – 15 mg/L. A target level of 15 – 20 mg/L for MRSA or under microbiology advice.
- If levels are below target on the highest dose (1.5 g BD) discuss case with pharmacy or microbiology.

Table 3. Interpretation of vancomycin levels and maintenance dose adjustments

Pre-dose (trough level)	How to adjust the maintenance dose given in table 2
Less than 5 mg/L	Move up two rows from current dosing schedule
5 – 10 mg/L	Move up one row
10 – 15 mg/L	Continue at current dose. (If target levels are 15 – 20 mg/L move up one row)
15 – 20 mg/L	Continue at current dose
20 – 25 mg/L	Move down one row without omitting any doses
25 – 30 mg/L	Omit next dose and move down two rows
More than 30 mg/L	Seek advice from microbiology

ADULT (over 18 years old) INTRAVENOUS VANCOMYCIN PRESCRIPTION CHART

<u>Patient Addressograph</u> Hospital No: Name: D.O.B:	Ward:	Consultant:
	Height (feet. inches):	Indication:
	Actual Body Weight (kg):	Target Level (circle): 10 – 15 mg/L <i>or</i> 15 – 20 mg/L
	Adjusted Body Weight (kg):	

Administration: See overleaf (front page). Maximum rate for infusion: 10 mg/minute. Speak to your pharmacist or consult the Injectable Medicines Guide (Medusa) for more information.

[illegible]